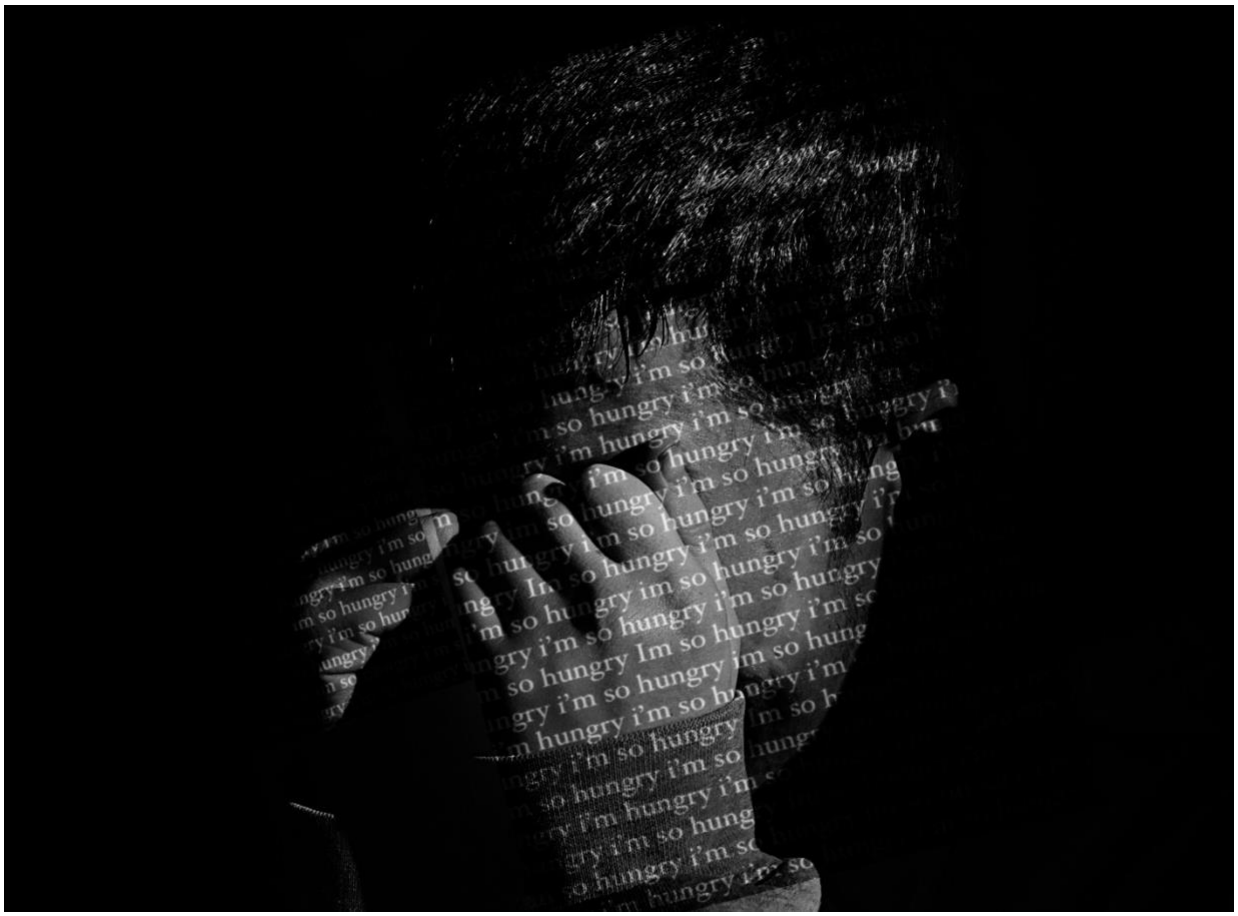


It Was Never Just Hunger

One Mother's Fight, a Community's Breakthrough, and a Future Once Impossible to Imagine

By Kate Fletcher

———

Most people think of hunger as a passing discomfort—something quiet and manageable. But in the world of Prader-Willi Syndrome (PWS), hunger is a relentless force. It doesn't whisper. It screams. It doesn't pause after meals or fade with time. It persists like panic trapped in the body—wired deep into the brain. For the people who live with it, and the families trying to help them survive it, hunger becomes a kind of haunting.

But PWS isn't just about hunger. It's about what happens when your child's biology betrays them. It's the invisible grief of a future that will never unfold, and the exhausting, tender devotion to the one that still can. It's sleepless nights, public meltdowns, emotional whiplash, and the kind of parenting that requires you to hold your ground when everything else is slipping.

"Not because I thought it would fix everything. But because for once, it felt like someone saw him. Someone saw us."

For decades, there was no treatment. Most doctors had never heard of it. Families were left cobbling together strategies through Facebook threads and trial-and-error. And then, this March, something changed. The FDA approved diazoxide choline extended-release—DCCR—the first drug developed to target the core symptom of PWS: hyperphagia. It was a milestone years in the making, pushed forward not just by scientists, but by families who fought to be seen.

"I cried when I read the news," says Kristin Reese, a mother in South Carolina whose 22-year-old son, Shields, lives with PWS. "Not because I thought it would fix everything. But because, for once, it felt like someone saw him. Someone saw us."

When Hunger Is the Enemy

There are clinical definitions of Prader-Willi Syndrome—words like "chromosome 15 deletion" and "hypothalamic dysfunction"—but for Kristin Reese, those terms have always felt too small. What matters more is the day-to-day reality of raising a child whose brain tells him, constantly and convincingly, that he is starving.

"Shields will eat dinner and immediately ask what's next," she says. "And if I say nothing, it's not just disappointment. It's panic. It's grief. It's like telling someone underwater they're not allowed to come up for air."

That sensation—known as hyperphagia—is one of the cruelest symptoms of PWS. It's not a strong appetite or emotional eating. It's a physiological signal that never shuts off. Shields's body can be full, but his brain tells him he's starving. Every single time.

When he hears "no," it doesn't land as a boundary—it lands as a threat. He begs. He pleads. Sometimes he cries, sometimes he freezes. Sometimes the panic turns to rage. Sometimes, one missing snack sends him into hours of dysregulation. It's not behavioral. It's not defiant.

It's what happens when the brain's most basic survival system misfires—and never stops.

"He's not throwing a tantrum," Kristin says. "He's drowning."

That's the part people don't see. Teachers. Doctors. Strangers in the checkout line. When Shields was younger, the misunderstandings were hard. Now that he's an adult, they can be dangerous. The bigger he gets, the more threatening his distress appears. The fear is constant. The explanations never seem to reach far enough.

And then there's the toll it takes at home.

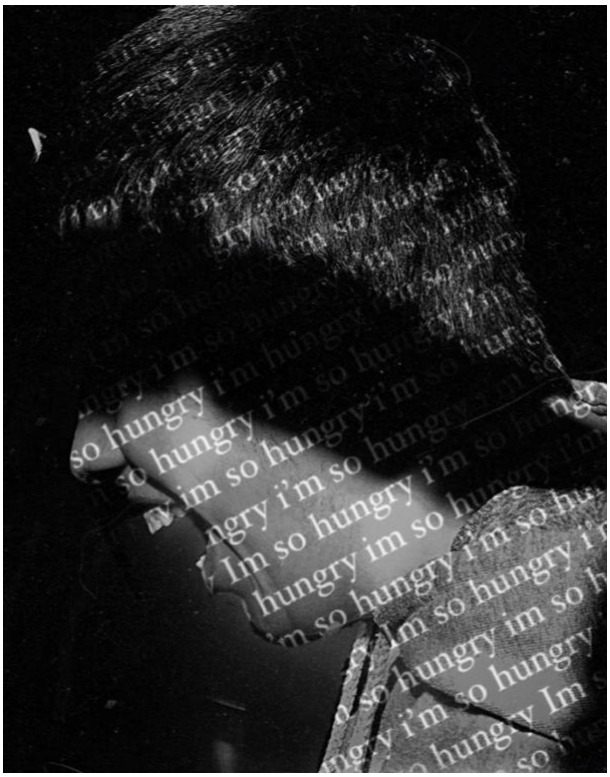
As Shields entered adolescence, the emotional volatility escalated. Hormonal changes deepened the hunger, shortened his fuse, and made even the best days unpredictable. Kristin was already holding things together with precision and exhaustion when the COVID-19 lockdowns began. Suddenly, the outside world disappeared—and the pressure inside their home multiplied.

"We were trapped," she says. "He was growing, the hunger was louder, the emotions were bigger, and there was nowhere to go."

Shields's younger sister was entering puberty too, trying to make sense of her own changing body while witnessing her brother unravel daily.

"She isolated more. She questioned her relationship with food. I think she felt scared. And guilty. She didn't say it, but I could see it," Kristin describes.

By then, even everyday moments became battlegrounds. *Mealtimes. Transitions. Public outings.* Kristin began to measure every plan against the potential fallout.



Images 1 and 2 taken by me. Depiction of the struggle of endless hunger in individuals with PWS.

"It's not that we didn't want to take him places. We just couldn't. Not when everything came with the risk of a meltdown. Not when we didn't have the energy left to recover from one," she continues.

And still—like so many parents raising kids with PWS—she kept showing up. Again and again. Finding new routines. Trying new strategies. Loving him fiercely in a world that rarely made space for either of them.

"I know my son's heart," she says. "He is kind. He is brilliant in ways the world doesn't always see. But for years, the hunger has been louder than all of that."

A New Kind of Waiting

For years, Kristin followed every update about DCCR like it was a breadcrumb trail through the dark. It wasn't a cure, and it wasn't even originally developed for Prader-Willi Syndrome, but it was the first medication that seemed to touch the root of the problem—the hunger itself. Clinical trial data showed improvement in hormone regulation, particularly ghrelin and leptin, the messengers responsible for hunger and satiety. But it wasn't just about the numbers. Families were reporting something even harder to quantify: peace.

It was subtle at first. A child sitting still longer. A tantrum that didn't come. A meal that ended without panic. One girl in the trial reportedly finished dinner, looked up, and said, "This is the first time I've felt full." Kristin couldn't forget that. Eight words, almost quiet. But they carried a kind of promise she had never dared imagine.

In early 2020, that promise landed on her doorstep. Shields was approved for the trial. There was a site within driving distance. It wasn't easy to get to, but it was possible. For the first time in a long time, Kristin let herself picture what relief might look like.

And then reality crept back in.

Shields was entering puberty, the most volatile phase of his life so far. Every symptom sharpened. The hunger grew louder, his emotional regulation more brittle, his reactions more explosive. Even short drives to local doctor appointments had become traumatic. He'd scream. He'd lash out. Sometimes he grabbed the steering wheel, suddenly and without warning.

"He's not throwing a tantrum," Kristin says. "He's drowning."

Kristin drove with white knuckles, tears in her eyes, trying to stay calm, trying to survive the next five minutes. "It wasn't just hard," she says. "It was dangerous. And terrifying. And constant."

They tried everything—calming tools, visual timers, breaks, rewards, weighted vests. Nothing changed the fact that getting Shields into a car and across the state line for a clinical trial would require both of them to survive a kind of emotional crucible just to arrive. And then do it again. And again. And again.

Kristin made the decision she didn't want to make.

She said *no.*

Not because she didn't believe in the drug. Not because she didn't want help. But because she

couldn't ask her son to endure one more thing just to get to the thing that might help.

And because she wasn't even sure if they would make it through the drive.

"I've never once regretted it," she says. "But I still grieve it. All the time."

A few weeks later, the pandemic hit.

Trial sites closed. Travel stopped. The world shrank in on itself, and suddenly the kind of isolation families like hers had always known became universal. The DCCR study was paused indefinitely, and even the families who had made it in were left waiting. Kristin watched from the outside as the program slowly restarted in 2021.

She followed every update. She read every email and every new round of data. Not with bitterness—but with the same quiet hope that had been there from the beginning. That maybe, eventually, it could circle back to Shields.

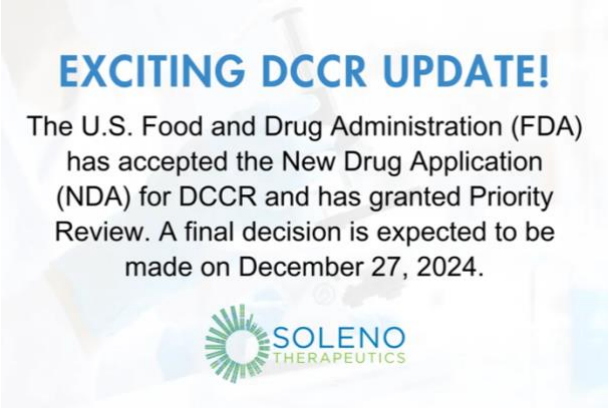


*Image courtesy of **Soleno Therapeutics**, retrieved from Google Images.*

In 2024, the drug's developer, *Soleno*, submitted DCCR for FDA approval. The agency granted it Priority Review.

Kristin held her breath. A decision was expected by the end of the year. Families wrote letters. Doctors rallied behind it. There was momentum.

Then came the delay:

Three more months.

No explanation. No concerns flagged. Just more waiting—for families already living at the edge of what they could bear.

And just as the silence settled in, something else shifted. Donald Trump returned to office. Within weeks, federal programs supporting rare disease research were slashed. The FDA lost thousands of employees in sweeping layoffs. Fast-track boards were dismantled. Review systems were gutted. And while no one directly linked the delay in DCCR's approval to the administration, the timing stung. For a community used to being overlooked, it felt like something more intentional. More targeted.

"We've been left behind before," Kristin says. "But this time, it felt deliberate. We weren't just rare—we were inconvenient."

Still, no one gave up. The PWS community has never had the luxury of silence. They mobilized again. Emails. Petitions. Testimonies. Reminders that this wasn't just about red tape. It was about a child who had never eaten a meal without fear. A parent running on empty. A family stretched thin by something they couldn't see and couldn't escape.

And then, finally, the decision came.

On March 26, 2025, DCCR was approved. It would be called ***VYKAT XR***—the first FDA-

approved treatment for hyperphagia in individuals with PWS.

Kristin didn't celebrate. She didn't cry right away.

She just sat still.

"I needed to feel what it meant," she says. "Not because I thought it would fix everything, but because someone finally said, 'We see you.' And that was enough."

Now, a new kind of waiting begins.

Will insurance cover it? Will doctors understand how to prescribe it? Will pharmacies stock it? Will Shields even qualify?

And beyond those questions, there's the broader uncertainty of navigating a system already being dismantled around her.

Kristin doesn't expect a miracle.

She's seen too much, lost too much, waited too long.

But she allows herself a cautious kind of hope—a hope she hasn't felt in years. Maybe this will be the thing that softens the edge. Maybe it will help her son breathe a little easier. Maybe, for once, they won't have to fight so hard to just exist.

"I don't need it to change everything," she says. "But if it can take the edge off his pain—even a little—that's everything."